

FEVER RESPONSE CARD



What counts as a fever: 100.4°F (38°C) or higher — rectal is most accurate under 3 months

Written by a Family Nurse Practitioner | Based on AAP & Bright Futures Guidelines

CHILD'S AGE	TEMPERATURE	ACTION TO TAKE
Under 2 months	ANY 100.4°F+	CALL or GO TO ER immediately — do not wait, even at night
2 months – 2 years	Under 104°F, acting OK	Call pediatrician same day
2 months – 2 years	Over 104°F or very ill	Call immediately or go to the ER now
Over 2 years	Under 104°F, alert & drinking	Monitor at home — call if fever lasts more than 3 days
Any age	105°F or higher	Go to ER now — do not wait

TREAT FOR COMFORT — not to chase a number

- **Acetaminophen (Tylenol)**
Ages 2 months+ — dose by WEIGHT, not age
- **Ibuprofen (Motrin/Advil)**
Ages 6 months+ — dose by WEIGHT, not age
- **Never give aspirin**
Not safe for any child under any circumstance
- Dress lightly — do not bundle
- Offer fluids frequently
- Lukewarm bath for comfort only

GO TO THE ER RIGHT NOW IF:

- Under 2 months with ANY fever — always an emergency
- Child won't wake up or is very hard to rouse
- Lips or fingernails look bluish
- Difficulty breathing or fast breathing at rest
- Stiff neck, bad headache, or light sensitivity
- Rash that does NOT turn white when pressed
- Seizure of any kind — even a first one

REMEMBER:

A higher fever does not always mean a more serious illness.

A playful child with 104°F can be less worrying than a limp, unresponsive child with 101°F. Watch the child — not the number.

WEIGHT-BASED FEVER MEDICATION GUIDE

Always dose by your child's **CURRENT WEIGHT** — never by age alone

Confirm concentration on your bottle before each dose | When in doubt, call your pediatrician

Acetaminophen (Tylenol)

Every 4–6 hours as needed | Ages 2 months+ | Max 5 doses in 24 hours

Ibuprofen (Motrin / Advil)

Every 6–8 hours as needed | Ages 6 months+ | Give with food or milk

Weight (lbs)	Weight (kg)	Tylenol Infant 160 mg/5 mL	Tylenol Children's 160 mg/5 mL	Motrin Infant 50 mg/1.25 mL	Motrin Children's 100 mg/5 mL
6–11	2.7–5	1.25 mL	—	—	—
12–17	5.5–7.7	2.5 mL	—	—	—
18–23	8–10.5	3.75 mL	—	1.25 mL	—
24–35	11–15.9	5 mL	5 mL	1.875 mL	5 mL
36–47	16–21	—	7.5 mL	—	7.5 mL
48–59	22–26	—	10 mL	—	10 mL
60–71	27–32	—	12.5 mL	—	12.5 mL
72–83	33–37	—	15 mL	—	15 mL
84–95	38–43	—	17.5 mL	—	17.5 mL

ACETAMINOPHEN (TYLENOL) NOTES

- Infant drops (160 mg/5 mL) are same concentration as Children's liquid — confirm label before dosing
- Safe from 2 months if fever is present
- Do not use for pain only under 2 months without MD guidance
- Max 5 doses in any 24-hour period

IBUPROFEN (MOTRIN/ADVIL) NOTES

- Not safe for infants under 6 months old
- Always give with food or milk to protect stomach
- Do not use if child is vomiting or dehydrated
- Infant drops: 50 mg/1.25 mL | Children's: 100 mg/5 mL
- Max 4 doses in any 24-hour period

NEVER give aspirin to a child for any reason.

Do not alternate Tylenol and Motrin on a set schedule unless your provider directs you to.

Treat for comfort — a fever doing its job is completely normal.

SICK CHILD: CALL vs. GO TO ER vs. WATCH AT HOME

Use this card when you're not sure what to do — and trust your gut. If something feels very wrong, go in.

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SYMPTOM	WATCH AT HOME	CALL PEDIATRICIAN	GO TO ER NOW
FEVER	Child over 2 yrs, under 104°F, alert, drinking	Any fever under 2 yrs, fever over 3 days, not improving	Under 2 months with any fever, over 105°F, seizure
VOMITING	Under 24 hrs, holding some fluids, no blood	Over 24 hrs in infant; 48 hrs in older child; repeated in newborn	Blood in vomit, extremely lethargic, signs of dehydration
DIARRHEA	Mild, staying hydrated, no blood	More than 5 loose stools/day, signs of mild dehydration	Blood or mucus in stool, severe dehydration, infant under 3 months
CROUP	Mild barking cough, calm child, breathing comfortably	Worsening overnight, not improved with cool air or steam	Stridor at rest, retractions, bluish lips, drooling
EAR PAIN	Mild tugging, no fever, acting okay	Fever with ear pain, no improvement in 48 hrs on antibiotics	Severe pain with high fever, ear drainage, stiff neck
RASH	Mild hives, no breathing trouble, rash turns white when pressed	Spreading rash, hives with mild swelling, rash after fever breaks	Rash that does NOT turn white when pressed — ER now
BREATHING	Mild congestion, breathing normally at rest	Fast breathing, noisy breathing not croup, persistent wheezing	Retractions, blue lips, grunting, can't speak or cry, stridor

DEHYDRATION WARNING SIGNS — Act Quickly:

- No wet diaper in 8 hrs
- No tears when crying
- Dry / cracked lips
- Sunken eyes or fontanelle
- Extremely sleepy, hard to wake

SICK DAY KIT CHECKLIST



Build your kit before you need it — so you're never searching at 2 AM

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MEDICATIONS & TREATMENTS

- ☐ Acetaminophen (Tylenol) — infant or children's
- ☐ Ibuprofen (Motrin/Advil) — ages 6 months+
- ☐ Oral rehydration solution (Pedialyte) — keep 1 on hand always
- ☐ Nasal saline drops or spray
- ☐ Antihistamine (ask your pediatrician for rec)
- ☐ Hydrocortisone 1% cream — for mild rashes/itching
- ☐ Petroleum jelly — for chapped skin, diaper barrier

Check expiration dates every 6 months

TOOLS & DEVICES

- ☐ Digital thermometer (rectal for infants, temporal/oral for toddlers)
- ☐ Bulb syringe or NoseFrida nasal aspirator
- ☐ Cool-mist humidifier (cleaned weekly)
- ☐ Measuring syringe or dropper for medications
- ☐ Heating pad — low setting, never unsupervised
- ☐ Disposable gloves (for illness cleanup)

Replace thermometer batteries each fall

COMFORT SUPPLIES

- ☐ Extra pacifiers (sanitized and ready)
- ☐ Lightweight blanket — for fever, dress lightly
- ☐ Favorite comfort item / stuffed animal
- ☐ Clear broth or bland snacks (crackers, applesauce)
- ☐ Extra change of clothes within easy reach

EMERGENCY NUMBERS — FILL IN NOW

Pediatrician main line:

Pediatrician after-hours:

Nearest children's ER:

Nearest urgent care (kids):

Poison Control (national):

1-800-222-1222

KNOW YOUR CHILD'S INFO

- ☐ Current weight (needed for dosing)
- ☐ Blood type (if known)
- ☐ Allergies — medications and foods
- ☐ Current medications and doses
- ☐ Insurance card photo saved to phone
- ☐ Pediatrician's fax number (for records)

FEEDING DURING ILLNESS

- ☐ Pedialyte / oral rehydration — small sips, often
- ☐ Breastfeed on demand — continue through illness
- ☐ BRAT foods: banana, rice, applesauce, toast
- ☐ Avoid juice and sports drinks for young infants
- ☐ Popsicles — help with hydration and sore throat
- ☐ When to reintroduce normal foods: after 6 hrs no vomiting

WHEN TO KEEP HOME FROM DAYCARE / SCHOOL

- ☐ Fever 100.4°F+ — home until 24 hrs fever-free without medication
- ☐ Vomiting or diarrhea — home until 24 hrs after last episode
- ☐ Rash without a doctor's note clearing return to care
- ☐ Strep throat — home until 24 hrs on prescribed antibiotics
- ☐ Pink eye with discharge — until treatment has been started

WHEN IN DOUBT — CALL YOUR PEDIATRICIAN.

No question is too small. That is what your pediatrician is there for.

Trust your gut — you know your child